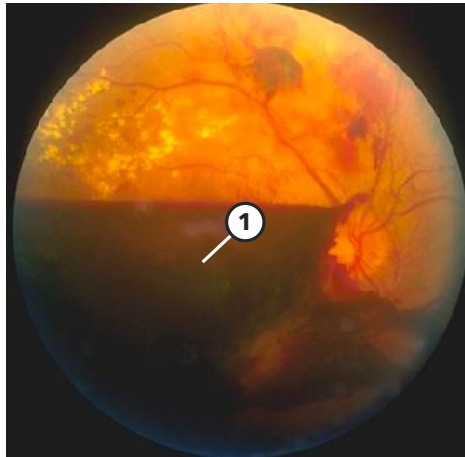


Retinal blood: locate it, then find its source

Clinical M3-M5 | Focused Type B notes

The vessel relationship gives depth

A preretinal fluid level



1: boat-shaped blood with a horizontal upper edge. Underlying retinal vessels are obscured by the collection. This localises the blood in front of the retina. Supplied M3 Anki, media 196.

Blood can occupy different compartments

Vitreous side above; RPE / choroid side below

PRERETINAL



INTRARETINAL



SUBRETINAL



Retina (teal) | RPE (gold) | Blood (red)

Original depth schematic: preretinal, intraretinal and subretinal blood have different relationships to retinal vessels.

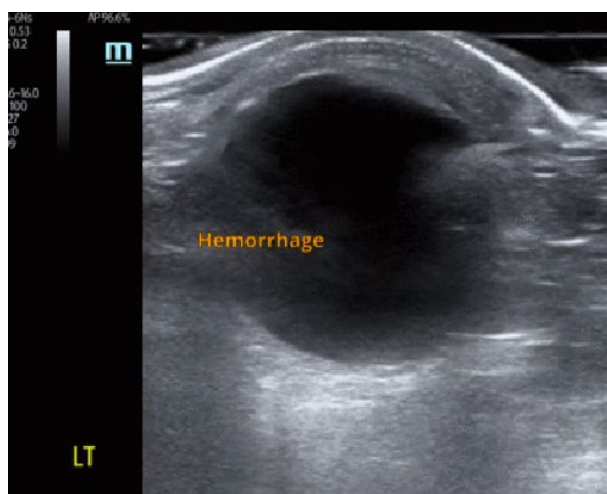
Compartment is not a complete diagnosis

Preretinal or vitreous blood can arise from fragile retinal new vessels or a tear, and other causes exist. Integrate the full fundus, history and retinal examination rather than inferring PDR from the shape alone.

References AAO: Vitreous haemorrhage | AAO: Diabetic Retinopathy PPP, 2025

An obscured fundus leaves uncertainty

Ultrasound can assess behind opaque vitreous



Supplied B-scan example of vitreous haemorrhage. Ultrasound helps detect detachment through opaque media, but a small retinal tear may be missed. Supplied M3 Anki, media 215.

A negative scan does not finish the examination

- 1 Assess acuity, pupils and anterior segment; investigate the haemorrhage source.
- 2 Use ultrasound when the retina cannot be adequately seen.
- 3 Maintain close retinal review and complete peripheral examination as the view clears; consider vitreoretinal intervention when indicated.

Do not anchor on diabetes

An acute haemorrhage with flashes can reflect a PVD-associated tear even in a patient with diabetes. Document which retinal areas were actually visualised and give clear urgent-return advice.